Capecitabine + temozolomide (CAPTEM)

NCCP 00505 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Dose and meal timing

The source starting schedule is capecitabine 750 mg/m² twice daily on Days 1–14 and temozolomide 200 mg/m² once daily on Days 10–14 of a 28-day cycle. Your own label gives the actual tablet/capsule numbers. Take capecitabine with food or within 30 minutes afterwards; take temozolomide fasting according to the pharmacy instructions. Swallow both whole; do not crush, cut or open them.

STEP 2

Missed or vomited doses

Do not double capecitabine or take a replacement dose after vomiting. If you miss temozolomide, follow the pharmacy's written instructions or call before taking it late. If you vomit after temozolomide, do not take a second dose that day. Never replace doses omitted because of toxicity.

STEP 3

Support medicines

Have the Days 10–14 anti-sickness prescription ready before Day 10. Ask how it fits around fasting temozolomide and meals for capecitabine. Follow the local rescue and constipation/diarrhoea plan.

STEP 4

Medicine interactions

Bring a complete medicine list. Warfarin may need extra INR checks. Tell the team about seizure medicines and recent brivudine or related antiviral treatment; dangerous interactions require specialist review.

STEP 5

Storage and handling

Keep each medicine in its own labelled original packaging and away from children. Do not cut or crush capecitabine or open temozolomide capsules. If swallowing is difficult, contact pharmacy before treatment. Return unused medicine as advised.

STEP 6

Pregnancy, breastfeeding and fertility

These medicines can harm a pregnancy and may affect fertility. Discuss contraception for patients and partners, the required duration after the last dose, fertility preservation and breastfeeding before starting; follow the medicine-specific written plan.

STEP 7

Review appointments

Before starting: blood count, kidney/liver tests, hepatitis B screening and DPD testing unless already tested. Blood count, kidney and liver tests before every cycle; liver tests also after each cycle. INR if indicated with warfarin. Liver injury can appear weeks after the last dose. Scans and hormone markers follow the consultant plan.

Sources: NCCP 00505 v5 · current Irish CAPTEM regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.